

The Lancet Regional Health – Americas is a new open access journal, part of *The Lancet's* global initiative to advocate for health-care quality and access in all regions of the world. The journal publishes high-quality original research that advocates change in, or illuminates, clinical practice and health policy in the Americas. We also consider relevant reviews, commentaries, and opinion pieces. The journal invites submissions that are pertaining to regional health subjects, including but not limited to infectious diseases, non-communicable diseases, child and adolescent health, maternal and reproductive health, emergency care, health policy, and health equity.

Manuscript preparation must adhere to relevant reporting standards on EQUATOR network website ([Enhancing the Quality and Transparency of Health Research](#)). Further details on the different sections of *The Lancet Regional Health – Americas*, and how to submit to the journal, are provided below. If you require further clarification, the journal's editorial staff will be pleased to help (americas@lancet.com).

Manuscripts must be solely the work of the author(s) stated, must not have been previously published elsewhere, and must not be under consideration by another journal. *The Lancet* journals are signatories of the [Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals](#), issued by the International Committee of Medical Journal Editors (ICMJE Recommendations), and to the Committee on Publication Ethics (COPE) code of conduct for editors. We follow [COPE's guidelines](#).

How to submit your paper

Manuscript submission

Manuscript submission to all *Lancet* journals is free. Payment of article processing fees is made after acceptance (see later). Manuscripts should be submitted online via the *The Lancet Regional Health – Americas's* online submission and peer review website (known as EM) at <https://www.editorialmanager.com/tlrhamericas/>

- Simply log on to EM and follow the on-screen instructions for all submissions
- If you have not used EM before, you will need to register first. In EM, the corresponding author is the person who enters the manuscript details and uploads the submission files
- Inclusion of illustrations (eg, photographs, graphs, diagrams) is a prerequisite for many publication types. Submission of original and editable artwork files is encouraged. Digital photography files should have a resolution of at least 300 dpi and be at least 107 mm wide. Before and after images should be taken with the same intensity, direction, and colour of light
- In almost all cases, if you have a finished manuscript, you should submit it, rather than contacting *The Lancet Regional Health – Americas* to enquire whether an unseen manuscript is likely to be accepted. Unless you have been asked by the Editor to submit by email, you should use the online system for all types of submission
- If you have any technical problems or questions, please contact our dedicated journal office inbox at editorial@lancet.com, the editor at americas@lancet.com, or visit our [Support Center](#) for further assistance

Covering letter

- You should upload your covering letter at the "Enter Comments" stage of the online submission process
- Use the covering letter to explain why your paper should be published in *The Lancet Regional Health – Americas* rather than elsewhere

Statements, permissions, and signatures

Authors and contributors

- Designated authors should meet all four criteria for authorship

First submissions to *The Lancet Regional Health – Americas* should include:

- 1 Covering letter
- 2 Manuscript including tables and panels
- 3 Figures
- 4 Author statement form (see next section)
- 5 Declaration of interests and source of funding statements (see next section)
- 6 In-press papers—one copy of each with acceptance letters
- 7 Protocols and CONSORT details for randomised controlled trials (see Articles)
- 8 We encourage disclosure of correspondence from other journals and reviewers, if previously submitted, and we might contact relevant editors of such journals
- 9 Research in context panel, for all primary research Articles

in the ICMJE Recommendations

- All authors, and all contributors (including medical writers and editors), should specify their individual contributions at the end of the text
- We require that more than one author directly accessed and verified the underlying data reported in the manuscript at the level of the individual participant. For research articles that are the result of an academic and commercial partnership, at least one of the authors named as having accessed and verified data must be from the academic team. The contributors statement should state who those authors are.
- All authors should confirm that they had full access to all the data in the study and accept responsibility to submit for publication
- We encourage collaboration and coauthorship with colleagues in the locations where the research is conducted
- The *Lancet* Group takes a neutral position with respect to territorial claims in institutional affiliations
- When choosing coauthors, we ask lead authors to be mindful of the benefits of diversity in authorship and to consider inviting coauthors who reflect diversity in every sense, including (but not limited to) background, career-stage, gender, geography, race, ethnicity, and (dis)ability
- We encourage authors of papers focused on minoritised

Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals
<http://www.icmje.org>

COPE Core Practices
<https://publicationethics.org/core-practices>

populations to include authors from those populations. Authors are also encouraged to describe whether and how people with lived experience of minoritisation were involved in or led their research.

- *The Lancet Regional Health – Americas* will not publish any paper unless we have the signatures of all authors
- We suggest you use the [author statement form](#) and upload the signed copy with your submission
- Please include written consent of any cited individual(s) noted in acknowledgments or personal communications
- For author groups of more than 30 members, we encourage use of a collaborator or study group for any additional authors. For this collaborator or study group, if they wish to be indexed to the paper, please provide a separate document with a table of first names and surnames of all members of the group (this is to ensure that PubMed and similar databases encode the names correctly).

Author statement form
<https://www.thelancet.com/forms?journal=lanam>

Changes to authorship

Changes to authorship once a manuscript has been submitted will only be possible in exceptional circumstances. It is important that authors carefully consider the authorship list and order of authors and provide a definitive author list at original submission. All authors must be listed in the manuscript and their details entered into the submission system. Any addition, deletion or rearrangement of author names in the authorship list should only be made prior to acceptance, and only if approved by the journal editor. Requests to change authorship should be made by the corresponding author, who must provide the reason for the request to the journal editor with written confirmation from all authors, including any authors being added or removed, that they agree with the addition, removal or rearrangement. Only in exceptional circumstances will the journal editor consider the addition, deletion or rearrangement of authors post acceptance.

Reporting sex-based and gender-based analyses

Reporting guidance

For research involving or pertaining to humans, animals, model organisms, or eukaryotic cells, investigators should integrate sex-based and gender-based analyses into their research design according to evolving funder/sponsor requirements and best practices within a field. Authors should address their research's sex and/or gender dimensions in their manuscript. In cases where they cannot, they should discuss this as a limitation to their research's generalisability. With research involving cells and model organisms, researchers should use the term "sex". With research involving humans, researchers should consider which terms best describe their data (see Definitions section below). Authors can refer to the [Sex and Gender Equity in Research \(SAGER\) Guidelines](#) and the [SAGER guidelines checklist](#). They offer systematic approaches to the use and editorial review of sex and gender information in study design, data analysis, outcome reporting, and research interpretation. However, there is no single, universally agreed-upon set of guidelines for defining sex and gender or reporting sex-based and gender-based analyses.

Definitions

In human research, the term "sex" carries multiple definitions. It often refers to an umbrella term for a set of biological attributes

associated with physical and physiological features (eg, chromosomal genotype, hormonal levels, internal and external anatomy). It can also signify a sex categorisation, most often designated at birth ("sex assigned at birth") based on a newborn's visible external anatomy. The term "gender" generally refers to socially constructed roles, behaviours, and identities of women, men, and gender-diverse people that occur in a historical and cultural context, and might vary across societies and over time. Gender influences how people view themselves and each other, how they behave and interact, and how power is distributed in society. Sex and gender are often incorrectly portrayed as binary (female/male or woman/man), concordant, and static. However, these constructs exist along a spectrum that includes additional sex categorisations and gender identities, such as people who are intersex/have differences of sex development (DSD), or identify as non-binary. In any given person, sex and gender might not align, and both can change. Sex and gender are not entirely discrete concepts and their definitions continue to evolve. Biology and society influence both, and many languages do not distinguish between them. Since the terms "sex" and "gender" can be ambiguous, authors should describe the methods they use to gather and report sex-related and/or gender-related data (eg, self-report or physician-report, specific biological attributes, current sex/gender, sex assigned at birth, etc) and discuss the potential limitations of those methods. This will enhance the research's precision, rigor, and reproducibility, and avoid ambiguity or conflation of terms and the constructs to which they refer. Authors should use the term "sex assigned at birth" rather than "biological sex", "birth sex" or "natal sex" as it is more accurate and inclusive. When ascertaining gender and sex, researchers should use a two-step process: (1) ask for gender identity allowing for multiple options and (2) if relevant to the research question, ask for sex assigned at birth. In addition to this defining guidance and the SAGER guidelines, you can find further information about reporting sex and gender in research studies in The Lancet Group's commitment to equity, diversity, and inclusion [here](#)

Reporting on race and ethnicity

We encourage researchers to include people from minoritised racial or ethnic populations as participants, and to plan to report and analyse data by race, ethnicity, or both. Disaggregating these data can help to uncover health inequities. In the Methods section, please explain the definitions, categories, or conceptual framework used and how they were assigned (eg, self-report, census or registry data). If specific data on race or ethnicity were not collected, analysed, or reported, it would be useful to give the reasons for this if possible.

Furthermore, for research specifically involving groups that have historically been marginalised, how have researchers prioritised community engagement and self-determination in the research process? For research involving Indigenous peoples, a possible tool to help you report this is the [CONSIDER statement](#).

In the Discussion section, please discuss the representativeness of the study population, to help readers assess the applicability of the findings to their setting.

Because race is a sociocultural construct, not a fixed biological trait, we ask authors to avoid use of race-based reference ranges and algorithms. For all manuscripts, any discussion of data in relation

Elsevier's author guide
<https://www.elsevier.com/en-gb/researcher/author/policies-and-guidelines/edi>

The Lancet Group's commitment to equity, diversity, and inclusion
<https://www.thelancet.com/equity-diversity-inclusion/commitments>

Sex and Gender Equity Research (SAGER) Guidelines
<https://researchintegrityjournal.biomedcentral.com/articles/10.1186/s41073-016-0007-6>

SAGER guidelines checklist
<https://ese.arphahub.com/article/86910/>

to race, ethnicity, or both should consider the wider context of socioeconomic, historical, and other structural drivers, for which race or ethnicity might be surrogate measures. We ask authors to qualify race-based associations drawn from observational data by discussing the potential limitations of such data and the possible role of unmeasured confounders. Such explanation can reduce the potential for harm from simplistic attributions to race.

We also ask authors to consider a **strengths-based approach** rather than a deficit discourse, when discussing findings related to race and ethnicity. For instance, discuss how findings might promote health and wellbeing, rather than focusing on problems.

Reporting on disability

We encourage researchers to include people with disabilities throughout the research process, from study design to collecting and reporting data on disability. Disability is an important determinant of health and access to services, and reporting data on disability, including disability-disaggregated data where feasible and appropriate to the study design, can help identify inequities.

Because disability reflects functioning in context, it cannot be reliably inferred from diagnosis or disease status alone. Authors should therefore avoid using the presence or absence of specific medical conditions as a proxy for disability and use validated measures of functioning where possible. When reporting disability-related data, authors should describe the methods used to identify disability, including the instrument or questions used, and how information was collected (eg, self-report, proxy report, administrative data). Authors should also discuss limitations of the measures used, including validation in local contexts and languages.

Researchers should also consider accessibility and inclusion throughout the research process, and authors should describe any measures taken to support accessibility and inclusion. Where eligibility criteria, study procedures, or other design features might exclude people with disabilities, authors should provide a clear rationale and discuss the implications on their findings.

Use of generative AI and AI-assisted technologies

Authors preparing a manuscript for an Elsevier journal can use AI tools to support them. However, these tools must never be used as a substitute for human critical thinking, expertise, and evaluation. AI tools should always be applied with human oversight and control. Ultimately, authors are responsible and accountable for the contents of their work. This includes accountability for:

- Carefully reviewing and verifying the accuracy, comprehensiveness, and impartiality of all AI-generated output (including checking the sources, as AI-generated references can be incorrect or fabricated).
- Editing and adapting all material thoroughly to ensure the manuscript represents the author's authentic and original contribution and reflects their own analysis, interpretation, insights, and ideas.
- Ensuring the use of any tools or sources, AI-based or otherwise, is made clear and transparent to readers—for the use of AI tools we require a disclosure statement upon submission.
- Ensuring the manuscript is developed in a way that safeguards data privacy, intellectual property, and other rights, by checking the terms and conditions of any AI tool that is used.

Authors must check the terms and conditions of any AI tool that they use to ensure that the privacy and confidentiality of their data and inputs, including their unpublished manuscripts, is maintained. Particular care should be taken with any personally identifiable data. Images that duplicate or refer to existing copyrighted images, real people, or others' identifiable products or brands must not be generated, nor any likeness of an individual's voice. Authors should check for factual errors and for any potential bias.

Authors should also check the terms and conditions of any AI tool they wish to use to ensure that they only grant to the AI tool the right to use their materials to provide the service to them and that they do not grant to the AI tool any other rights to the materials that they input into the AI tool (including without limitation the right to train the AI tool on those materials). They must also ensure that the AI tool does not impose constraints on the use of outputs from the AI tool in a way that could restrict the subsequent publication of the relevant article.

Authors should disclose the use of AI tools for manuscript preparation in a separate AI declaration statement at the end of their manuscript upon submission and this statement will appear in the published work. Authors should document their use of AI, including the name and version of the AI tool used, the purpose of the use, and the extent of their oversight. Prompts should be provided where applicable and editors reserve the right to ask authors for these. Declaring the use of AI tools supports transparency and trust between authors, readers, reviewers, editors, and contributors and facilitates compliance with the terms of use of the relevant AI tool. Basic checks of grammar, spelling, and punctuation need no declaration. AI use in the research process should be declared and described in detail in the Methods section. Editors reserve the right to determine whether the use of AI is appropriate.

Authors should not list AI tools as an author or co-author, nor cite AI tools as an author. Authorship implies responsibilities and tasks that can only be attributed to and performed by humans.

For figures, generative AI should be used primarily as a tool for brainstorming and suggesting image concepts. Its direct use in creating figures or artwork should be limited to demonstrations of its capabilities in specific contexts, such as academic articles. This caution is due to potential copyright issues associated with using images from the internet without proper attribution.

Blue section content, which includes Comments and Correspondence, is a space in *The Lancet's* journals for human interpretation of science and medicine. For Comments and Correspondence, the use of AI-assisted tools and generative AI is permitted only to assist with grammar and spelling. AI-assisted tools must not be used to: translate Comments or Correspondence at the submission stage, in whole or in part; improve arguments; generate new content or sentences; reorder content; shorten or summarise content; alter meaning; or generate, complete, or verify references or citations. For Comments linked to Research Articles, no confidential unpublished data relating to the Article you are commenting on should be input into any AI-assisted tool. If you use AI to assist with grammar and spelling, you must remove any confidential information before using the tool. Any breach of this policy will lead to rejection of your Comment or Correspondence. If AI has been used, a declaration must be included at the submission stage that fully describes how you have used AI.

Strengths-based approach
<https://www.lowitja.org.au/wp-content/uploads/2023/05/deficit-discourse-strengths-based.pdf>

Reporting on disability
[https://www.thelancet.com/journals/lancet/article/PIIS0140-6736\(26\)01547-3/fulltext](https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(26)01547-3/fulltext)

Forms and signatures

For Comment and Correspondence, we require you to upload your forms at submission. For original research (Articles), we will request these forms after peer review. The following signed statements are required:

- [Author statements](#)
- [Conflicts of interest statements](#) (ICME forms)
- Statements of role, if any, of medical writer or editor
- Acknowledgments—written consent of cited individual
- Personal communications—written consent of cited individual
- Use of copyright-protected material—signed permission statements from author and publisher

These statements can be scanned and submitted electronically with your submission. Please note that *The Lancet* journals will accept hand-signed and electronic (typewritten) signatures.

Declaration of interests

A conflict of interest exists when professional judgement concerning a primary interest (such as patients' welfare or validity of research) may be influenced by a secondary interest (such as financial gain). Financial relationships are easily identifiable, but conflicts can also occur because of personal relationships or rivalries, academic competition, or intellectual beliefs. A conflict can be actual or potential, and full disclosure to the Editor of all relationships is a requisite. Purposeful failure to disclose conflicts is a form of misconduct and might lead to publication of a correction or even to retraction. All submissions to *The Lancet Regional Health – Americas* must include disclosure of all relationships in which there is a potential or actual conflict of interest, even if it not directly relevant to the submitted work. The Editor may use such information as a basis for editorial decisions and will publish all disclosures that authors declare on their conflict of interests form. It is the corresponding author's responsibility to check that all declarations made by authors on their conflicts of interest form are included at the end of the manuscript. Agreements between authors and study sponsors that interfere with authors' access to all of a study's data, or that interfere with their ability to analyse and interpret the data and to prepare and publish manuscripts independently, may represent conflicts of interest, and should be avoided. Authors may be required to provide the journal with any such agreements in confidence.

- At the end of the text, under a subheading "Declaration of interests", all authors must disclose any financial and personal relationships with other people or organisations, even if it does not directly relate to the submitted work. Examples of financial conflicts include employment, consultancies, stock ownership, honoraria, paid expert testimony, patents or patent applications, and travel grants, all within 3 years of beginning the work submitted. If there are no conflicts of interest, authors should state that none exist
- All authors are required to provide a Conflict of Interest Statement and should complete a standard form, which is available at <https://www.thelancet.com/forms?journal=lanam>. The form has been modified by the ICMJE following consultation with authors and editors. Further information is available in a joint ICMJE statement published on July 1, 2010. For more information see [Lancet 2009; 374: 1395–96](#).
- For any Comment, Review, Health Policy, Personal View, or

Viewpoint, *The Lancet Regional Health – Americas* will not publish if an author, within the past 3 years, and with a relevant company or competitor, has any stocks or shares, equity, a contract of employment, or a named position on a company board; or has been asked by any organisation other than *The Lancet Regional Health – Americas* to write, be named on, or to submit the paper (see [Lancet 2004; 363: 2–3](#))

Role of the funding source

- All sources of funding should be declared as an acknowledgment at the end of the text
- Within that "Acknowledgements" section, authors must describe the role of the study sponsor(s), if any, in study design; in the collection, analysis, and interpretation of data; in the writing of the report; and in the decision to submit the paper for publication
- If the funding source had no such involvement, the authors should state this

Role of medical writer or editor

- If a medical writer or editor was involved in the creation of your manuscript, we need a signed statement from the corresponding author to include their name and information about funding of this person
- This information should be added to the Acknowledgments or Contributors section
- We require signed statements from any medical writers or editors declaring that they have given permission to be named as an author, as a contributor, or in the Acknowledgments section

Patient and other consents

- Appropriate written consents, permissions, and releases must be obtained where you wish to include any case details, personal information, and/or images of patients or other individuals in *The Lancet* journals in order to comply with all applicable laws and regulations concerning privacy and/or security of personal information. Studies on patients or volunteers need approval from an ethics committee and informed consent from participants. These should be documented in your paper.
- Since the consent form needs to comply with the relevant legal requirements of your particular jurisdiction, we do not provide sample forms; this is your responsibility. Your affiliated institution should be able to provide an appropriate form.
- For the purposes of publishing in *The Lancet* journals, a consent, permission, or release should include, without limitation, publication in all formats (including print, electronic, and websites), in sublicensed and reprinted versions (including translations), and in other works and products.
- To respect your patient's and any other individual's privacy, please do not send signed forms to *The Lancet Regional Health – Americas*. Please instead complete the patient consent section of the [Author statements](#) while retaining copies of the signed forms in the event they should be needed.
- If consent, permission, or release is made subject to any conditions, *The Lancet Regional Health – Americas* must be made aware in writing of all such conditions before publication.

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- For more information about our policy, please visit <https://www.elsevier.com/about/our-business/policies/patient-consent>.

Types of article and manuscript requirements

Please ensure that anything you submit to *The Lancet Regional Health – Americas* follows the guidelines provided for each article type. For instruction on how to format the text of your paper, including tables, figures, panels, and references, please see our **Formatting guidelines**.

Articles

- The Lancet Regional Health – Americas* prioritises reports of original research that are likely to change practice or thinking
- We invite submission of all trials, whether phase 1, 2, 3, or 4. For phase 1 trials, we consider those of a novel substance for a novel indication, if there is a strong or unexpected beneficial or adverse response, or a novel mechanism of action
- We require the registration of all interventional trials, whether early or late phase, in a primary register that participates in [WHO's International Clinical Trial Registry Platform](#) (see *Lancet* 2007; 369: 1909–11) or in [ClinicalTrials.gov](#), in accord with [ICMJE recommendations](#). We also require full public disclosure of the minimum 24-item trial registration dataset at the time of registration and before recruitment of the first participant (see *Lancet* 2006; 367: 1631–35). The registry must be independent of for-profit interest
- Reports of trials must conform to [CONSORT 2025 guidelines](#), and should be submitted with a full protocol in English, or, with the full protocol and a synopsis in English including details of enrolment criteria, outcomes/endpoints, and statistical considerations. Please also submit a statistical analysis plan with the protocol if there is one
- All reports of randomised trials should include a section entitled Randomisation and masking, within the Methods section. Please refer to *The Lancet's* [formatting guidelines for randomised trials](#)
- Cluster-randomised trials must be reported according to [CONSORT extended guidelines](#)
- Randomised trials that report harms must be described according to [extended CONSORT guidelines](#)
- Randomised trials that include participants aged 0–19 years should be reported according to the [CONSORT-C extension guidelines](#)
- Studies of diagnostic accuracy must be reported according to [STARD guidelines](#)
- Observational studies (cohort, case-control, or cross-sectional designs) must be reported according to the [STROBE statement](#), and should be submitted with their protocols
- We encourage the registration of all observational studies on a WHO-compliant registry (see *Lancet* 2010; 375: 348)
- Genetic association studies must be reported according to [STREGA guidelines](#)
- Qualitative studies should be reported according to [COREQ](#) or [SRQR](#) guidelines, as applicable.
- Systematic reviews and meta-analyses must be reported according to [PRISMA guidelines](#). Please refer to *The Lancet's*

[formatting guidelines for systematic reviews and meta-analyses](#).

- Reports of studies of global health estimates should be reported according to the [GATHER statement](#) (see *Lancet* 2016; 388: e19–23)
- Clinical trials that report interventions using artificial intelligence must be described according to the [CONSORT-AI Extension guidelines](#) and their protocols must be described according to the [SPIRIT-AI Extension guidelines](#)
- To find reporting guidelines see: <http://www.equator-network.org>
- When using a study group, collaborator group, or Consortia instead of authors' names, please be aware that individuals' names will not explicitly appear when your published Article is uploaded to MEDLINE/PubMed. Your Article will still be discoverable via a search for a specific named author, but only the collective name given to the study will appear on that platform. If you need more information, please contact us.

All Articles should, as relevant:

- Be around 3500–5000 words with 30 references (the word count is for the manuscript text only)
- Qualitative and mixed-methods research articles can be more flexible regarding the total word count. The main text should be 4500 words and include a maximum of 30 references. Editors can guide you on additional word limits for quotes (unedited, to be presented in tables or panels). Please contact the editorial office at americas@lancet.com for more information.
- Include an abstract (semistructured summary), with five paragraphs (Background, Methods, Findings, Interpretation, and Funding), not exceeding 250 words. Our electronic submission system will ask you to copy and paste this section at the "Submit Abstract" stage
- For randomised trials, the abstract should adhere to CONSORT extensions: abstracts (see *Lancet* 2008; 371: 281–83)
- When reporting Kaplan-Meier survival data, at each timepoint, authors must include numbers at risk, and are encouraged to include the number of censored patients.
- For intervention studies, the abstract should include the primary outcome expressed as the difference between groups with a confidence interval on that difference (absolute differences are more useful than relative ones). Secondary outcomes can be included as long as they are clearly marked as secondary and all such outcomes are reported
- Use the recommended international non-proprietary name (rINN) for drug names. Ensure that the dose, route, and frequency of administration of any drug you mention are correct
- Use gene names approved by the [Human Gene Organisation](#). Novel gene sequences should be deposited in a public database (GenBank, EMBL, or DDBJ), and the accession number provided. Authors of microarray papers should include in their submission the information recommended by the [MIAME guidelines](#). Authors should also submit their experimental details to one of the publicly available databases: [ArrayExpress](#) or [GEO](#)
- Include any necessary additional data as part of your EM submission
- All accepted Articles should include a link to the full study protocol published on the authors' institutional website (see *Lancet* 2009; 373: 992 and *Lancet* 2010; 375: 348)

WHO's International Clinical Trial Registry Platform
<https://www.who.int/clinical-trials-registry-platform>

Clinical trials
<http://clinicaltrials.gov>

ICMJE recommendations
<http://icmje.org/recommendations/browse/publishing-and-editorial-issues/clinical-trial-registration.html>

CONSORT 2025 guidelines
<https://www.consort-spirit.org/>

Formatting guidelines for randomised trials
<https://www.thelancet.com/for-authors/forms?section=rct>

CONSORT extended guidelines
<http://www.consort-statement.org/extensions/extensions/>

CONSORT-C Extension guidelines
[https://doi.org/10.1016/S2352-4642\(26\)00004-0/](https://doi.org/10.1016/S2352-4642(26)00004-0/)

STARD guidelines
<http://www.stard-statement.org/>

STROBE statement
<http://www.strobe-statement.org/>

STREGA guidelines
<http://www.equator-network.org/reporting-guidelines/strobe-strega/>

COREQ or SRQR guidelines
<https://www.equator-network.org/reporting-guidelines/coreq/>
<https://www.equator-network.org/reporting-guidelines/srqr/>

PRISMA guidelines
<http://www.prisma-statement.org/>

Formatting guidelines for meta-analyses
<https://www.thelancet.com/for-authors/forms?section=meta-analysis>

GATHER statement
[http://www.thelancet.com/journals/lancet/article/PIIS0140-6736\(16\)30388-9/fulltext](http://www.thelancet.com/journals/lancet/article/PIIS0140-6736(16)30388-9/fulltext)

CONSORT-AI Extension guidelines
[https://doi.org/10.1016/S2589-7500\(20\)30218-1](https://doi.org/10.1016/S2589-7500(20)30218-1)

SPIRIT-AI Extension guidelines
[https://doi.org/10.1016/S2589-7500\(20\)30219-3](https://doi.org/10.1016/S2589-7500(20)30219-3)

To find reporting guidelines, see <http://www.equator-network.org>

Human Gene Organisation
<http://www.genenames.org/>

MIAME guidelines
<https://www.ncbi.nlm.nih.gov/info/MIAME.html>

Array and GEO

<http://www.ebi.ac.uk/microarray-as/ae/>

<http://www.ncbi.nlm.nih.gov/geo>

- We encourage researchers to enrol women and ethnic groups into clinical trials of all phases, and to plan to analyse data by sex and by race

Putting research into context

- All research papers (including systematic reviews/meta-analyses) submitted to *The Lancet Regional Health – Americas* must include a panel putting their research into context with previous work in the format outlined below (see [Lancet 2014; 384: 2176–77](#), for the original rationale). This panel should not contain references. Editors will use this information at the first assessment stage and peer reviewers will be specifically asked to check the content and accuracy
- The Discussion section should contain a full description and discussion of the context. Authors are also invited to either report their own, up-to-date systematic review or cite a recent systematic review of other trials, putting their trial into context of the review

MENDELEY data

<https://data.mendeley.com>

Research in context

Evidence before this study

This section should include a description of all the evidence that the authors considered before undertaking this study. Authors should briefly state: the sources (databases, journal or book reference lists, etc) searched; the criteria used to include or exclude studies (including the exact start and end dates of the search), which should not be limited to English language publications; the search terms used; the quality (risk of bias) of that evidence; and the pooled estimate derived from meta-analysis of the evidence, if appropriate.

Added value of this study

Authors should describe here how their findings add value to the existing evidence.

Implications of all the available evidence

Authors should state the implications for practice or policy and future research of their study combined with existing evidence.

Research in context panels should not contain references; key studies mentioned here should be referenced in the main text.

Data sharing

All submitted research Articles must contain a data sharing statement, to be included at the end of the manuscript. Data sharing statements must include:

- Whether data collected for the study, including individual participant data and a data dictionary defining each field in the set, will be made available to others (“undecided” is not an acceptable answer);
- What data will be made available (deidentified participant data, participant data with identifiers, data dictionary, or other specified data set);
- Whether additional, related documents will be available (eg, study protocol, statistical analysis plan, informed consent form);
- When these data will be available (beginning and end date, or “with publication”, as applicable);
- Where the data will be made available (including complete URLs or email addresses if relevant);
- By what access criteria data will be shared (including with whom,

for what types of analyses, by what mechanism – eg, with or without investigator support, after approval of a proposal, with a signed data access agreement – or any additional restrictions).

See [table](#) for examples. Clinical trials that begin enrolling participants on or after Jan 1, 2019, must include a data sharing plan in the trial's registration. If the data sharing plan changes after registration, this should be reflected in the statement submitted and published, and updated in the registry record. [Mendeley Data](#) is a secure online repository for research data, permitting archiving of any file type and assigning a permanent and unique digital object identifier (DOI) so that the files can be easily referenced. If authors wish to share their supporting data, and have not already made alternative arrangements, a Mendeley DOI can be referred to in the data sharing statement.

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The Lancet Regional Health – Americas encourages the submission of translated summaries (abstracts) in languages that are relevant to the country where the research was done. Translated summaries are published unedited and unformatted, as a separate supplementary file. If your paper is accepted, we will offer you the opportunity to prepare a translation of the final edited summary. We do not require translated material at submission stage.

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Editorials are the voice of *The Lancet Regional Health – Americas*, and are written in-house by the journal's editorial-writing team and signed “*The Lancet Regional Health – Americas*”. Editorials (and other short opinion pieces such as Comment and Correspondence) written by in-house editors are critiqued and revised by the editorial team. They are not subject to external peer review.

Comment

- This section contains commentaries that accompany papers published in *The Lancet Regional Health – Americas*, or to issues of wide-reaching concern in medical research and health policy. Most commentaries are commissioned, but unsolicited commentaries are also welcome. Commentaries may be peer reviewed
- Commentaries should be no more than 750 words, 10 references, and one figure, panel, or small table
- See **Conflicts of Interest** guidelines for comments

Correspondence

- We welcome correspondence on content published in *The Lancet Regional Health – Americas* or on other topics of interest to our readers
- Letters for publication in the journal must reach us within 8 weeks of the online publication date of the original item and should be no longer than 400 words
- Letters of general interest, unlinked to items published in the journal, can be up to 400 words long
- Letters are not usually peer reviewed, but they will be subject to editorial scrutiny before any decision to publish is made; we might consult authors of the original publication for advice; and occasionally invite formal replies from the authors of the original publication for inclusion in the journal alongside a letter
- Only one table or figure is permitted, and there should be no more than five references and five authors

- All accepted letters are edited, and proofs will be sent out to authors before publication

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- Any substantial error in any article published in *The Lancet Regional Health – Americas* should be corrected as soon as possible. Blame is not apportioned; the important thing is to set the record straight.
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Reviews

Most reviews are commissioned, but unsolicited short outlines (300–400 words) can be directed to the Editor. If you have already written the paper, please submit it for consideration via our [online system](#)

- Reviews should be either a definitive overview of a major topic connected with regional health or an update of knowledge in a somewhat narrower field of current interest
- Manuscripts will be assessed in-house and those judged suitable will be peer reviewed before an editorial decision is made
- Reviews should be no more than 4500 words, with a maximum of 75 references
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- A 150-word unstructured summary should be included. Use of up to 5–6 illustrations is encouraged to aid the reader
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Search strategy and selection criteria

References for this Review were identified through searches of PubMed with the search terms “young onset”, “early onset”, “presenile”, and “dementia” from 1995 until April, 2019. Articles were also identified through searches of the authors’ own files. Only papers published in English were reviewed. The final reference list was generated on the basis of originality and relevance to the broad scope of this Review

- Systematic reviews should be prepared according to the PRISMA guidelines

Health Policy

- Manuscripts considered for this section are narrative reviews (not original research) and should follow the same guidelines as a Review (no more than 4500 words, with a maximum of 75 references, and a 150-word unstructured Summary)
- These papers should cover developments in regional health related to policy, guideline development, health systems, or economics. Other related topics may be considered

Personal View

- Personal View papers should follow the same guidelines as a Review (no more than 4500 words, with a maximum of 75 references, and a 150-word unstructured Summary)
- These opinion pieces encourage debate and allow opinions to be expressed on a topic of immediate relevance to regional health. The authors should use the best evidence to develop a robust argument

Viewpoint

- Viewpoint papers are no more than 2500 words, with a maximum of 30 references
- The authors should have a novel and clear point to make, with the aim of provoking transformational thinking on issues related to regional health

Series

- Series are collections of papers (usually between three and five) on a clinical or regional health topic. They are generally commissioned by the editors, but suggestions are welcome by email.

Formatting guidelines

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Title page

- A brief title, author name(s), preferred degree (one only), affiliation(s), and full address(es) of the authors must be included. The name and address of the corresponding author should be separately and clearly indicated along with email and telephone details

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- Type a single space at the end of each sentence
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- Do not worry about type of font or point size
- We use a comma before the final “and” or “or” in a list of items
- Type decimal points midline (ie, 23·4, not 23.4). To create a midline decimal on a PC: hold down ALT key and type 0183 on the number pad, or on a Mac: ALT shift 9
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- Do not use the automated features of your software, such as hyphenation, endnotes, headers, or footers (especially for references). Please use page numbering

References

- Cite references in the text sequentially in the Vancouver numbering style, as a superscripted number after any punctuation mark. For example:
 "...as reported by Saito and colleagues.¹⁵"
- Two references are cited separated by a comma, with no space. Three or more consecutive references are given as a range with an en rule. To create an en rule on a PC: hold down CTRL key and minus sign on the number pad, or on a Mac: ALT hyphen
- References in tables, figures, and panels should be in numerical order according to where the item is cited in the text
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"...15[tab]Saito N, Ebara S, Ohotsuka K, Kumeta J, Takaoka K. Natural history of scoliosis in spastic cerebral palsy. *Lancet* 1998; **351**: 1687-[en rule]92."

- Give any subpart to the title of the article. Journal names are abbreviated in their standard form as in [Index Medicus](#)
- If there are six authors or fewer, give all six in the form: [surname] [initials]...
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Drug names
 For more on neuroscience-based nomenclature see
[http://www.thelancet.com/pdfs/journals/lanpsy/PIIS2215-0366\(17\)30098-6.pdf](http://www.thelancet.com/pdfs/journals/lanpsy/PIIS2215-0366(17)30098-6.pdf)

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- Do not use titles in the graph or artwork. Titles should appear at the beginning of the figure legend.
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- Numbers in text and tables should always be provided if % is shown
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- p values should be given to two significant figures, unless p<0.0001

Drug names

- Recommended international non-proprietary name (rINN) is required

- We encourage use of neuroscience-based nomenclature for psychotropic drugs

References

- Vancouver style—eg,
Smith A, Jones B, Clements S. Clinical transplantation of tissue-engineered airway. *Lancet* 2008; **372**: 1201–09.
- Hourigan P. Ankle injuries. In: Chan D, ed. *Sports medicine*. London: Elsevier, 2008: 230–47.
- Numbered in order of mention in appendix and numbered separately from references in the full paper

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